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Bridging Gaps in Care: Strategic Policy Initiatives for Enhancing Health Care Services for Aging Women

Prepared for:
National Women's Health Network
Raaya Alim, MPP
Policy Associate

Chairs: Sophia C. Scott '25 and Thomas A. Tait '26

TLs: Christopher Y. Shin '27, Bolu Ilelaboye '25, and Alvira Tyagi '25

Researchers: Chinedum Aguwa, Fhasal Alam, Dana Garibaldi Saenz, Daniella Elge, Neeraja Kumar, Aneesh Mazumder, and Tsering Yangchen

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This report is the result of a semester-long intensive effort by a team of undergraduates. The Institute of Politics does not endorse specific policy positions; accordingly, all views expressed in this publication should be understood to be solely those of the authors.

ABOUT THE NATIONAL WOMEN'S HEALTH NETWORK

The National Women's Health Network is a 501c3 not for profit that represents the health interests of women across the life continuum.

It maintains an intersectional focus on sexual and reproductive health, maternal health and the health and well-being of aging women.

The NWHN works to improve women's health outcomes through state and federal advocacy, consumer health education, and grassroots technical assistance initiatives.

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Institute of Politics
Harvard University
79 John F. Kennedy Street
Cambridge, MA 02138
Tel: (617) 495-1360
Fax: (617) 496-4344
Web: www.iop.harvard.edu
Email: policy@harvardiop.org

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Institute of Politics
Harvard University
79 John F. Kennedy Street
Cambridge, MA 02138
Tel: (617) 495-1360
Fax: (617) 496-4344
Web: www.iop.harvard.edu
Email: ioppolicy@gmail.com

Table of Contents

Executive Summary **5**
Physical Health and Wellness **6**
Recommendations 6
Establish a Concrete Menopause Curriculum in U.S. Medical Schools 6
Lobby for Expansion of the Protect Black Women and Girls Act of 2021 6
Lobby Congress to Pass the Menopause Research and Equity Act of 2023 7
Lobby Congress to Pass the NIH Clinical Trial Diversity Act of 2023 7
Language Policy 8
Supporting Evidence 9
Establish a Concrete Menopause Curriculum in U.S. Medical Schools 9
Lobby for Expansion of the Protect Black Women and Girls Act of 2021 10
Lobby Congress to Pass the Menopause Research and Equity Act of 2023 11
Lobby Congress to Pass the NIH Clinical Trial Diversity Act of 2023 13
Language Policy 14
Sexual Health **16**
Recommendations 16
Implement Humanistic, Patient-Focused Sexual Health Checklist 16
Institute Specialized Sexual Health Workshops in Publicly-Funded Initiatives 16
Urge the Right to Emergent Menopausal Paid Leave-of-Absences 17
Develop a Standardized Sexual Health Curriculum that Incorporates Aging Women 17
Broader Implementation of Telemedicine for Sexual Health Consultations in Rural United States 18
Supporting Evidence 18
Implement Humanistic, Patient-Focused Sexual Health Checklist 18
Institute Specialized Sexual Health Workshops in Publicly-Funded Initiatives 20
Urge the Right to Emergent Menopausal Paid Leave-of-Absences 21
Develop a Standardized Sexual Health Curriculum that Incorporates Aging Women 22
Broader Implementation of Telemedicine for Sexual Health Consultations in Rural United States 23

Executive Summary

Aging women in the United States are one of the most vulnerable demographic groups in terms of receiving adequate healthcare. While biological, socioeconomic, social, and environmental factors have significant impacts on women's aging, little consideration is given to health issues that this vulnerable population faces. This has manifested itself in numerous problems in terms of the management and delivery of healthcare for aging women. Diseases that disproportionately affect women are extremely understudied. For many of these diseases, treatment options can be significantly higher compared to diseases that exclusively affect men, resulting in a financial barrier that hinders aging women from getting proper treatment. Furthermore, certain sexual diseases have been stigmatized for the aging women population. In conjunction with all of these issues, little legislative effort has been done in order to remedy these issues. As these issues continue to progress, it is imperative for targeted legislation and interventions to occur in order to reduce the disparities in care that aging women receive.

From February to May 2024, the Harvard Kennedy School Institute of Politics Health Policy Team researched ways to effectively address issues regarding the healthcare management and delivery for aging women in collaboration with the National Women's Health Network. We segmented our investigation into three areas: (1) physical health and wellness, and (2) sexual health. This framework allows us to obtain an all-encompassing perspective on the issues regarding healthcare management and delivery for aging women and to galvanize different stakeholder groups. This brief outlines and advocates for a set of actions by providing evidence-based recommendations to address issues regarding the aging women population.

Physical Health and Wellness

Recommendations

Establish a Concrete Menopause Curriculum in U.S. Medical Schools

The National Women's Health Network should work with the Liaison Committee on Medical Education (LCME) to develop a concrete menopause curriculum for medical schools in the United States, modeling that proposed by the European Menopause and Andropause Society (EMAS). Currently, U.S. medical education lacks depth in education when it comes to addressing menopause, and this results in physicians being uninformed on menopause and other women-specific conditions. Thus, women are not getting adequate care. Addressing menopause during a physician's time in medical school will make the topic much more accessible for women when they do see their Primary Care Physician (PCP). Therefore, by this cultural barrier at the physician-level through a tailored curriculum about menopause, medical practitioners will have the proper knowledge regarding menopause and will be able to effectively provide adequate care for women going through menopause and other women-specific conditions.

Lobby for Expansion of the Protect Black Women and Girls Act of 2021

The National Women's Health Network should lobby for the expansion of H.R. 6268, the Protect Black Women and Girls Act of 2021, so that clinics can offer free or low-cost mammograms to the public. One of the big tenets of this bill is to advocate for local mammogram access for black women, which is something that has been inaccessible in the past. However, the current barriers still disproportionately impact black women, making these services still inaccessible. Therefore, by lobbying for local mammogram access for black women for free

or at low-costs, this can eliminate the barrier in aging black women's health that prevents them from receiving proper care for their medical needs.

Lobby Congress to Pass the Menopause Research and Equity Act of 2023

The National Women's Health Network should lobby Congress to pass the Menopause Research and Equity Act of 2023. This work would build off of President Biden's work on increasing women's health research funding. This bill would refocus the National Institute of Health to conduct and support research regarding the effects of menopause on middle-aged women's health and provide a report that includes strategies to address these concerns and gaps that further research could supplement. Since there is currently a lack of research regarding these medical conditions, lobbying for this bill would emphasize the necessity of this work in order to improve the quality of care aging women receive. The United States Senate Special Committee on Aging should oversee the bill's implementation to ensure the appropriate inclusion of the funds. This strategy would especially align with the National Women's Health Network's work, given its mission to improve health for aging women.

Lobby Congress to Pass the NIH Clinical Trial Diversity Act of 2023

The National Women's Health Network should lobby Congress to pass S.1701, the NIH Clinical Trial Diversity Act of 2023, to address the issues and barriers associated with including women in clinical trials. This bill focuses on providing resources to increase the diversity of participants in clinical trials and provide awareness for the trials and calls for the NIH to mandate plans for recruitment and retention of participants in the trial to receive funding. Additionally, the bill calls for specific communication from the NIH about the demographic data of clinical trial participants. The National Women's Health Network can also work with members of Congress to

provide resources to promote the inclusion of women specifically in the bill. The bill has already been referred to the United States Senate Committee on Health, Education, Labor, and Pensions. Therefore, if it passes, they should oversee its implementation within the NIH. By lobbying for this bill, the disparities that affect the research regarding aging women's health conditions can be significantly reduced.

Language Policy

Not all language assistant services are equivalent. The effectiveness of language assistance services is rarely analyzed and especially not for the most vulnerable female elderly population. For those who are elderly there needs to be a clear perspective on what is considered an effective language assistant service. Cognitive abilities can often decrease with age as well as hearing. Due to the demographics of the United States, this phenomenon is more prevalent among aging women than men. A language barrier on top of this already delicate situation is a setting for disaster and unnecessary human loss. Interpretive technology is not a warranted substitute for employment diversity in provision of language support services. Actually, there are findings in literature that doctor usage of google translate may be better than technological language translators in the provision of care. Having a live medical professional who can speak a particular language on staff at the hospital is essential to better care. This happens only by supporting diversity in hospital employment similar to the provisions of the Massachusetts Health Care system. Other recommendations include enhancing or regulating medical education where students are required to learn a different language medically as part of their training. A study of medical Spanish found that only 66% medical students offer Medical Spanish with a

curriculum, which is often an elective.[1] Increasing the languages taught in this capacity and ensuring that every school has a medical language problem can be very beneficial in also addressing this health gap.

Supporting Evidence

Establish a Concrete Menopause Curriculum in U.S. Medical Schools

One of the largest barriers women face in receiving menopause-based healthcare is the cultural barrier associated with women-specific diseases. This causes women going through menopause to be culturally shunned, which can lead them to feel ashamed and not seek medical help for potentially detrimental menopause symptoms. In many cultures, menopause is seen as a loss of youth, with words like "fertility failure" being used to describe it, and it is associated as a period where women lose sexual attractiveness.[2]

This cultural stigma surrounding menopause is exacerbated by current healthcare management of women-specific conditions, mainly due to a lack of comprehensive education. Currently, many U.S. medical schools do not go deeper into menopause than a lecture or two, which leaves many physicians in the dark as to how to treat common symptoms of menopause.[3] Due to how closely menopause is related to various conditions related to women's aging, a lack of knowledge on menopause can be detrimental to aging women's health conditions.[4] Because of this, the European Menopause and Andropause Society (EMAS) introduced a thorough

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[1] Morales, Raymond, Lauren Rodriguez, Angad Singh, Erin Stratta, Lydia Mendoza, Melissa A Valerio, and Monica Vela. "National Survey of Medical Spanish Curriculum in U.S. Medical Schools." Journal of general internal medicine, October 2015. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4579233/.

[2] Namazi, Masoumeh, Rasoul Sadeghi, and Zahra Behboodi Moghadam. "Social Determinants of Health in Menopause: An Integrative Review." *International Journal of Women's Health* 11 (December 9, 2019): 637–47. https://doi.org/10.2147/IJWH.S228594.

[3] Naidorf, Joan. "What They Don't Teach in Med School about Menopause." *Menopause Matters: Empowering Women's Health* (blog), September 18, 2023. https://medium.com/menopause-matters-empowering-womens-health/what-they-dont-teach-in-med-school-about-menopause-2a80327c054b.

[4] Rees, Margaret, Kathy Abernethy, Gloria Bachmann, Silvia Bretz, Iuliana Ceausu, Fatih Durmusoglu, Risto Erkkola, et al. "The Essential Menopause Curriculum for Healthcare Professionals: A European Menopause and Andropause Society (EMAS) Position Statement." *Maturitas* 158 (April 1, 2022): 70–77. https://doi.org/10.1016/j.maturitas.2021.12.001.

curriculum that could be implemented into medical schools and involves focusing on menopause and postmenopausal health, including assessment and diagnosis strategies.[5] This report provides detailed instructions for how doctors should treat diseases that are related to menopause, such as osteoporosis and decreased thyroid function. Overall, this curriculum is able to effectively address the cultural stigma that is associated with conditions related to women's aging, and therefore foster effective and informed communication with aging women and their physicians regarding their health conditions.

Lobby for Expansion of the Protect Black Women and Girls Act of 2021

Breast cancer is a health issue that especially impacts aging women, with most breast cancers being found in women aged 50 and older.[6] Black women are a vulnerable group of breast cancer patients, as they have a 40% higher breast cancer mortality rate compared to White women.[7] A lack of private health insurance contributes as much as 37% to this increased mortality rate.[8] Even for Black women on Medicaid, a study of 44 states found that in 30% of the states, Black women were significantly less likely to undergo breast cancer screening.[9] Another study of women on Medicare aged 40-89 found that mammography technologies were harder to access for Black women compared to White women.[10] Coupled with the fact that aging women often have fewer retirement funds compared to aging men — $57,000 compared to $118,000 —

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[5]Rees, Margaret, Kathy Abernethy, Gloria Bachmann, Silvia Bretz, Iuliana Ceausu, Fatih Durmusoglu, Risto Erkkola, et al. "The Essential Menopause Curriculum for Healthcare Professionals: A European Menopause and Andropause Society (EMAS) Position Statement." *Maturitas* 158 (April 1, 2022): 70–77. https://doi.org/10.1016/j.maturitas.2021.12.001.

[6] Centers for Disease Control and Prevention. "What Are the Risk Factors for Breast Cancer?," July 31, 2023. https://www.cdc.gov/cancer/breast/basic_info/risk_factors.htm#:~:text=Risk%20Factors%20You%20Cannot%20Change&text=The%20risk%20for%20breast%20cancer,of%20breast%20and%20ovarian%20cancer.

[7] Breast Cancer Research Foundation. "Black Women and Breast Cancer: Why Disparities Persist and How to End Them." Breast Cancer Research Foundation, April 4, 2024. https://www.bcrf.org/blog/black-women-and-breast-cancer-why-disparities-persist-and-how-end-them/.

[8] Jemal, Ahmedin, Anthony Robbins, Chun Chieh Lin, W. Dana Flanders, Carol DeSantis, Elizabeth Ward, and Rachel A. Freedman. "Factors That Contributed to Black-White Disparities in Survival Among Nonelderly Women With Breast Cancer Between 2004 and 2013." *Journal of Clinical Oncology* 36, no. 1 (January 1, 2018): 14–24. https://doi.org/10.1200/jco.2017.73.7932.

[9]"Access to Care as a Barrier to Mammography for Black Women." Policy, Politics & Nursing Practice/Policy, Politics, & Nursing Practice 22, no. 1 (October 19, 2020): 28–40. https://doi.org/10.1177/1527154420965537.

[10]"Relationship Between Race and Access to Newer Mammographic Technology in Women With Medicare Insurance." Radiology 306, no. 2 (February 1, 2023). https://doi.org/10.1148/radiol.221153.

aging Black women may not have enough funds to diagnose their breast cancer or seek high-quality treatment if they are diagnosed.[11] Despite policy developments in Medicaid expansion and the establishment of health programs to improve breast cancer detection, little is known about how these policies impact Black womens' ability to access mammography screenings, an important factor in their breast cancer journey that could impact their mortality.[12]

One bill that has been introduced into Congress in order to provide more access to mammograms for black women is H.R. 6268, the Protect Black Women and Girls Act of 2021.[13] It was introduced by Representative Kelly Robin of the Democratic Party, and its primary goal is to advocate for local mammogram access for Black women. Biennial mammogram access has proven to be effective in reducing the mortality rates of aging Black women with breast cancer. While this bill begins to address these systemic issues related to the disparities in aging black women's health, barriers regarding the accessibility to these services still remain an issue due to their associated costs. Eliminating financial barriers like these can result in eliminating differences in healthcare quality that disproportionately impact Black women's physical health.

Lobby Congress to Pass the Menopause Research and Equity Act of 2023

Though recent pushes have been made to increase the quality of women's healthcare as well as expand access to reproductive care, this has caused many women, specifically those who can no longer reproduce, to feel like their health has been put on the sidelines. A recent study done by *Nature Aging* has highlighted that 99% of studies on aging do not properly consider menopause, which is very concerning to note as menopause is something most women

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[11] TransAmerica. "Life in the COVID-19 Pandemic: Women's Health, Finances, and Retirement Outlook," October 2021. https://transamericainstitute.org/docs/default-source/research/women-retirement-security-report.pdf.

[12] "Access to Care as a Barrier to Mammography for Black Women." Policy, Politics & Nursing Practice/Policy, Politics, & Nursing Practice 22, no. 1 (October 19, 2020): 28–40. https://doi.org/10.1177/1527154420965537.

[13] Kelly, Robin. "H.R.6268 - Protect Black Women and Girls Act of 2021." Congress.Gov, November 1, 2022. https://www.congress.gov/bill/117th-congress/house-bill/6268.

experience.[14] Examples like this have led to major gaps in research and healthcare for many older women.

Healthcare systems worldwide lack data on how to treat age-related diseases in women. This has manifested itself in various forms. Since the hormonal effects of menopause are not taken into consideration as women age, they are more likely to develop conditions such as osteoporosis when compared to men.[15] Currently, there is no legislation that has passed that requires research or health organizations to conduct research on menopause, so the health of aging women is continuing to be sidelined. Due to this, aging women and their physician care team are left with many unanswered questions when it comes to their treatment.

Furthermore, funding for women's health research has not been aligned with the disease burden for women. In an analysis of the distribution of research funding from the National Institute of Health (NIH), applied mathematician Arthur Mirin, as reported by *Nature News*, found that the diseases that predominantly affected women were underfunded.[16] Diseases including migraine, headaches, and anxiety disorders disproportionately affect women, yet they don't receive funding in proportion to the severity of the issues for women.[16] [17]
Some of the other diseases that affect women to a large extent are Alzheimer's and cardiovascular disease (CVD).[18] Though two-thirds of Alzheimer's patients are women, a majority of the studies that include animal models don't report the sex of the animals they are studying.[18] In clinical trials for CVD, about a third of the participants are female, even though the

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[14] Gilmer, Gabrielle, Zachary R. Hettinger, Yetsa Tuakli-Wosornu, Elizabeth Skidmore, Julie K. Silver, Rebecca C. Thurston, Dawn A. Lowe, and Fabrisia Ambrosio. "Female Aging: When Translational Models Don't Translate." Nature Aging 3, no. 12 (December 2023): 1500–1508. https://doi.org/10.1038/s43587-023-00509-8.
[15] nhs.uk. "Osteoporosis - Causes," October 23, 2017. https://www.nhs.uk/conditions/osteoporosis/causes/.
[16] Smith, Kerri. "Women's Health Research Lacks Funding – These Charts Show How." Nature News, Nature Publishing Group, www.nature.com/immersive/d41586-023-01475-2/index.html. Accessed 27 Apr. 2024.
[17] Arthur A. Mirin, "Gender Disparity in the Funding of Diseases by the U.S. National Institutes of Health," *Journal of Women's Health (2002)* 30, no. 7 (July 2021): 956–63, https://doi.org/10.1089/jwh.2020.8682.
[18] Bird, Chloe E. "Underfunding of Research in Women's Health Issues Is the Biggest Missed Opportunity in Health Care | RAND." RAND, 11 Feb. 2022, www.rand.org/pubs/commentary/2022/02/underfunding-of-research-in-womens-health-is sues-is.html.

disease is the leading cause of death for women in the US. The unequal representation of women in these studies risks creating biased treatment profiles.

A study by the Women's Health Access Matters nonprofit organization analyzed the potential impact in terms of return on investment of allocating a larger budget toward women's health issues through simulations.[19] For example, in Alzheimer's Disease, they simulated the effects of increasing the budget from $288 million to $576 million.[19] The study found the return on investment of the funding to be very high, even when making conservative assumptions on the extent to which the funding would lead to health improvement.[19] One case they looked at was doubling the NIH budget for coronary artery disease from $20 million. In the simulation, they found that the return on investment would be 9,500 percent.[19] The study's results highlight the reality of funding for women's health research and the potential to increase the budget.

Lobby Congress to Pass the NIH Clinical Trial Diversity Act of 2023

Though work has been done to improve inclusion in clinical trials, women are still underrepresented in clinical trials, accounting for 29–34% of participants.[20] This issue stems from the pharmaceutical industry's concerns about the effects on pregnancies and fertility.[5] The current demographics of clinical trials risk safety profiles being built on men and not representing female reactions to treatments.[20]

Although many policies have been oriented toward diversifying research initiatives, there still is a lack of generalizability in most of the research that happens in a biomedical context.[21] One major proponent of this issue is that most test subjects are men, so many results are only

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[19] Bird, Chloe E. "Underfunding of Research in Women's Health Issues Is the Biggest Missed Opportunity in Health Care | RAND." RAND, 11 Feb. 2022, www.rand.org/pubs/commentary/2022/02/underfunding-of-research-in-womens-health-is sues-is.html.

[20] Fultinavičiūtė, Urtė. "Sex and Science: Underrepresentation of Women in Early-Stage Clinical Trials." Clinical Trials Arena, 17 Oct. 2022, www.clinicaltrialsarena.com/features/underrepresentation-women-early-stage-clinical-trials/?cf-view.

[21] Llorens, Anaïs, et al. "Gender Bias in Academia: A Lifetime Problem That Needs Solutions." Neuron, vol. 109, no. 13, July 2021, pp. 2047–2074, www.sciencedirect.com/science/article/pii/S0896627321004177, https://doi.org/10.1016/j.neuron.2021.06.002.

valid for men but not necessarily representative of women.[22] An example of the unique biological differences and inhibiting factors of studies that succumb to this are menstrual cycle, pregnancy, and menopause, which are unique to women and aren't considered in studies that only analyze men. Thus, it is essential for a standardized framework for research that is supposed to concern the general population to be created so that research projects can ensure that they are helping the most people.[22] [23]

One major limiting factor in clinical research for women is the lack of consideration for different parts of a woman's life, such as menopause and pregnancy. As a result, most clinical trials are not flexible nor have the ability to adjust when these periods potentially overlap with clinical trials. Thus, clinical trials for women are primarily prevented, as there is a risk of halting the research. As a result, it is vital to create a flexible framework that enables studies to work around these barriers to create more engagement with women in healthcare research.[24]

Language Policy

The rising issue of the language barrier is largely unnoticed especially to a largely older, female demographic. But, language barriers are a dangerous phenomenon within the field of healthcare. In a famous case cited in the AMA journal of Ethics, the care of a 13 year old female patient that came in with stomach pain and was treated for and ended up with a ruptured appendix and septic shock is often referred to when discussing language barriers.[25] In another study, it was found that when patients who did not speak a local language were treated in a

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[22] Liu, Katherine A., and Natalie A. DiPietro Mager. "Women's Involvement in Clinical Trials: Historical Perspective and Future Implications." Pharmacy Practice, vol. 14, no. 1, 6 Mar. 2016, pp. 708–708, www.ncbi.nlm.nih.gov/pmc/articles/PMC4800017/, https://doi.org/10.18549/pharmpract.2016.01.708.
[23] McKiever, Monique, et al. "Challenges in Conducting Clinical Research Studies in Pregnant Women." Journal of Pharmacokinetics and Pharmacodynamics, vol. 47, no. 4, 18 Apr. 2020, pp. 287–293, https://doi.org/10.1007/s10928-020-09687-z.
[24] "A Policy Framework for Inclusive Research Design." Wellcome, 18 Jan. 2023, wellcome.org/news/policy-framework-inclusive-research-design. Accessed 16 Apr. 2024.
[25] "How Should Clinicians Respond to Language Barriers That Exacerbate Health Inequity?" *AMA Journal of Ethics* 23, no. 2 (February 1, 2021). https://doi.org/10.1001/amajethics.2021.109.

hospital, around 49% did not understand the medical language and the medical problems only seemed to rise from there.[26]

The 1964 Civil Rights Act continues to be the single most important piece of legislation for providing limited English proficient (LEP) patients with access to language assistant services. Congress passed the Civil Rights Act to ensure that federal money was not used to support discriminatory practices in government programs and activities. Looking at title IV of the act, the Supreme court has treated discrimination based on language as equivalent to national origin discrimination. Language interpretation and assistance is required to those in need.[27]

[26] Al Shamsi, Hilal, Abdullah G Almutairi, Sulaiman Al Mashrafi, and Talib Al Kalbani. "Implications of Language Barriers for Healthcare: A Systematic Review." Oman medical journal, April 30, 2020. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC7201401/.
[27] Chen, Alice Hm, Mara K Youdelman, and Jamie Brooks. "The Legal Framework for Language Access in Healthcare Settings: Title VI and Beyond." Journal of general internal medicine, November 2007. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC2150609/.

Sexual Health

Recommendations

Implement Humanistic, Patient-Focused Sexual Health Checklist

The National Women's Health Network should propose a bill to implement a checklist, ensuring primary care physicians (PCPs) in the United States ask sexual health questions pertaining to aging women. A checklist emphasizes the contribution of sexual activity to the patient's overall wellbeing; otherwise, providers may fail to address sexual health concerns the patient may fear to bring up. Such checklists should be introduced in training for PCPs and be used for all post-menopausal women, minimizing implicit bias that may hinder PCPs ability to engage in sexual health—nurses and nursing assistants may be informed to take action as well to address sexual health concerns of aging women. The sexual health checklist for aging women proves critical to uplift aging women's sexual health concerns, forming a core part of their overall well-being and addressing critical lifestyle concerns.

Institute Specialized Sexual Health Workshops in Publicly-Funded Initiatives

The National Women's Health Network should expand resolution H.Res.7 into law requiring women's healthcare centers and other government-funded women's health centers to explicitly allocate funds for aging women, creating initiatives to provide information for aging women on changes in their physical bodies. Informational workshops help to provide real-time questions menopausal women have about their bodies, and community tabling establishes core relationships between local clinics and community aging women to build a sense of interdependency and trust. Such initiatives should be prompt and be offered at appropriate times,

particularly evenings in the weekdays and Sunday afternoons, to fit with aging women's lifestyle schedules; the initiatives must also consider language barrier for immigrant aging women and accommodate for women who have disabilities, including hard-of-hearing. Specified funding for informational workshops and community tabling for aging women will enable them to better understand their own bodies and provide guidance in maintaining healthy sexual habits.

Urge the Right to Emergent Menopausal Paid Leave-of-Absences

The National Women's Health Network should expand on new policy H.R.6749 for menopause and the workplace, advocating for the importance of establishing set, clear work hours for which women are allowed paid leave-of-absences. Such work hours will help liberate menopausal women who experience pain without having the ability to take leave from work. Establishing a specified number of work hours, depending on the strenuous nature of the work, will help prevent the risk of putting the reproductive health of aging women at risk. Set, clear work hours and paid leave-of-absences into law is vital to ensuring that work labor does not jeopardize the reproductive health of aging women in a difficult, dynamic time for their bodies.

Develop a Standardized Sexual Health Curriculum that Incorporates Aging Women

The U.S. Department of Health and Human Services should develop a standardized curriculum of clinic-based practices and knowledge about the sexual health of aging women that medical schools and continuing education centers can incorporate into their sexual health curriculum. Many medical professionals receive non-standardized and inadequate training on sexual health in aging women. Evidence shows that knowledge level of healthcare professionals is a significant predictor of the frequency at which sexual health is addressed among aging women in the clinical setting. Curriculum would address sexual and menstrual disorders among

aging women, its manifestations in differing populations, existing treatments and their financial landscape, and the biopsychosocial aspects of the construction of sexuality. This would build confidence and rectify false assumptions about sexual health in aging women among healthcare professionals, ground knowledge in the clinical realm, and encourage inclusive practices for trans, intersex, non-binary, and gender diverse people.

Broader Implementation of Telemedicine for Sexual Health Consultations in Rural United States

The National Women's Health Network should work to increase the access of telemedicine for sexually transmitted infections (STI) and overall sexual health consultation in rural areas in the United States. Currently, one of the barriers for rural women is the social stigma associated with sexual activity, and this stigma is even heightened for aging women. This is exacerbated by the transportation issues associated with access to care in rural areas. The implementation of wider accessibility to telemedicine would decrease the stigma surrounding sexual health and increase access to safer sex supplies, testing kits, and more.

Supporting Evidence

Implement Humanistic, Patient-Focused Sexual Health Checklist

Three key issues arise from a lack of support system at the primary care provider (PCP) level for aging women regarding sexual health. Firstly, the PCP's own judgment often clouds their quality of care in engaging with older women. Research suggests that pre-existing "personal perceptions" creates a "barrier to addressing older women's sexual health concerns" as

physicians do not correlate sexual health with older women.[28] Secondly, attitudes towards the state of the patient provides a challenge in addressing sexual health concerns. Often, women will feel "shame or fear" in changes with your bodies, preventing them from speaking out to physicians.[19] Further, the lack of information surrounding sexual health post-menopause exacerbates the stigmatization surrounding sexuality, leading aging women to attribute "sexual problems to normal aging" and leading physicians to view older women's challenges towards a fruitful, intimate life as "not serious."[19] Thirdly, the patient experience at the physician's office provides an unfruitful, dichotomous situation when patients complete health forms. Health forms often pre-suppose the existence or absence of marital status, preventing the option for women to explain their sexual health situation regardless of marital status.

Further, the questions overemphasize treating sexual diseases rather than the overall sexual life of an older woman. One woman comments, "It was like filling out a driver's license form. There was no intimate piece. It was more like a legal document. Maybe it was used for the doctor to cover any health insurance issues. There was no area where I could write in or mark what my relationship was or sexual history was," expressing concerns over the lack of opportunities to explain her sexual health life.[29] Providing open-ended questions about sexual life helps to empower aging women, preventing them from feeling a sense of insecurity over their changing bodies following menopause. Education and training in Western countries showcase trends in biases surrounding aging women. Physicians tend to disregard the sexual health of aging women under the assumption that as women age, addressing intercourse concerns become

[28] Muliira, Joshua K., and Rhoda S. Muliira. 2013. "Sexual Health for Older Women : Implications for Nurses and Other Healthcare Providers = الصحة الجنسية للنساء المسنات : إنعكاساتها ممرضين و ممرضات و مقدمي الرعاية الصحية الأخرى." Sultan Qaboos University Medical Journal 13 (4): 469–76. https://doi.org/10.12816/0003304.
[29] Mary C. Politi et al., *Journal of General Internal Medicine* 24, no. 4 (April 2009): 511–16, https://doi.org/10.1007/s11606-009-0930-z.

more irrelevant. For example, in the United Kingdom, general practitioners (GPs) view sexual health as an illegitimate topic to discuss with older women post-menopause.[30]

The literature suggests changes in education and training on sexual health for GPs should change to alleviate the burden of patients' challenges in sexual health. Another case study showcases the use of checklists in the Medical Checklist Act of 2013 for health practices and organizations, particularly those seeking to "conduct a study on the development and efficacy of medical checklists" through rigorous modeling and implementation.[31] Thus, medical checklists provide tools for physicians to ensure they have not missed their checkup in sexual health for post-menopausal women, hindering pre-existing biases.

Institute Specialized Sexual Health Workshops in Publicly-Funded Initiatives

Doubt in sexual expression, as a result of changing bodies, hinder aging women from engaging with sexual health services. Firstly, research suggest older women engage in frequent sexual intercourse with partners, whether that would be marital marriages or short-term endeavors; yet, the dynamics of post-menopausal bodies present the possibility for women to have "difficulty with vaginal lubrication and anorgasmia," a type of delayed, infrequent orgasm in aging women.[32] Secondly, as a result of physical changes in aging women, patients may begin to feel changes in their self-expression. Particularly, upon surveying aging women across the United States, many felt that they did "not want to express [their] sexuality" out of shame and discomfort.[33] Challenges in self-expression also impact their relationships with their partners if

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[30] Merryn Gott, Sharron Hinchliff, and Elisabeth Galena, "General Practitioner Attitudes to Discussing Sexual Health Issues with Older People," *Social Science & Medicine* 58, no. 11 (June 2004): 2093–2103, https://doi.org/10.1016/j.socscimed.2003.08.025.https://doi.org/10.1016/j.socscimed.2003.08.025Gott, Hinchliff, and Galena.

[31] "H.R. 116 - Medical Checklist Act of 2013" (2013), https://www.congress.gov/bill/113th-congress/house-bill/116/all-info.

[32] Muliira, Joshua K., and Rhoda S. Muliira. 2013. "Sexual Health for Older Women : Implications for Nurses and Other Healthcare Providers = الصحة الجنسية للنساء المسنات : إنعكاساتها ممرضين و ممرضات و مقدمي الرعاية الصحية الأخرى." Sultan Qaboos University Medical Journal 13 (4): 469–76. https://doi.org/10.12816/0003304.

[33] Macleod, Ashley, Lucy Busija, and Marita McCabe. 2020. "Mapping the Perceived Sexuality of Heterosexual Men and Women in Mid- and Later Life: A Mixed-Methods Study." *Sexual Medicine* 8 (1): 84–99. https://doi.org/10.1016/j.esxm.2019.10.001.

their marital status exists; many aging women feel "energy levels" and "a fear of being hurt" in a intimate and sexual level with their partners arise from a lack of being able to express their sexuality fully.[24]

These barriers to sexual expression and intimacy prove crucial prior to searching for sexual health services; in fact, such barriers prevent aging women from taking on opportunities provided by existing health services. As a result, the challenges to free sexual self-expression for post-menopausal women give rise to fear of health services seeking to alleviate such barriers. Samping over 1,550 women between 57 to 85 years of age, data show that albeit the dynamic physical changes of post-menopausal women, "all elderly women engage in, or wish to engage in, sexual activity."[34] There is little evidence suggesting aging women engage in less sexual activity than younger generations; therefore, improving sexual health for aging women should be a priority for PCPs and healthcare professionals working with especially geriatric populations.

Urge the Right to Emergent Menopausal Paid Leave-of-Absences

The psychosocial implications of the workplace, beyond the stress of high-stakes jobs, proves to be dangerous for women undergoing menopause and postmenopausal symptoms. In fact, multiple survey-based studies have shown the detrimental effects of strenuous workplace environments on the sexual health of women, exacerbating pre-existing menopausal symptoms. In a cohort of aging women between the ages of 50-64, researchers surveyed 400 workers for symptoms; such symptoms included sudden flushes (colloquially "night flushes"), trouble sleeping, psychological distress, and urinary issues.[35] Coupling workplace environments causes risk factors for aging women, placing their sexual and mental health at risk. Beyond a required

[34] Stacy Tessler Lindau et al., "A Study of Sexuality and Health among Older Adults in the United States," *New England Journal of Medicine* 357, no. 8 (August 23, 2007): 762–74, https://doi.org/10.1056/NEJMoa067423.
[35] Stefania D'Angelo et al., "Impact of Menopausal Symptoms on Work: Findings from Women in the Health and Employment after Fifty (HEAF) Study," *International Journal of Environmental Research and Public Health* 20, no. 1 (December 24, 2022): 295, https://doi.org/10.3390/ijerph20010295.

paid leave-of-absence, research suggests that more awareness is needed "amongst employers in all sectors"; for women in financial difficulties, should the leave-of-absence be unpaid, it can exacerbate symptoms in that the aging women are forced to work as they struggle to manage their sexual health symptoms.

Develop a Standardized Sexual Health Curriculum that Incorporates Aging Women

While sexual liberation movements have constructed a more accepting climate to discuss sexual health, the vitality and youth associated with these movements have prevented this shift from permeating the realm of aging women. It remains a taboo to discuss menstrual health, menopause, STDs, among other sexual health disorders for aging women. In fact, many aging women do not believe sexual health symptoms such as vaginal pruritus and dryness to be a medical condition but rather a natural part of the aging process.[36] This develops the first barrier for addressing sexual health in the clinical space. Another is the negative perceptions that exist on certain treatments.

Underreporting and the absence of these discussions also occurs because of the health professional. Many gynecologists do not believe that sexual health is relevant amongst aging women or relevant to the wellbeing of the population[37] and those that do, rarely address or perform services for it.[38] A factor in this is time limitation. A study by the American College of Obstetricians and Gynecologists' Committee that 22.7% of professionals reported not discussing sexual health due to time constraints [39] This demonstrates how sexual health is deemed low on the list of priorities even when the patient may still have persistent sexual health symptoms.

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[36] Wong EL, Huang F, Cheung AW, Wong CK. The impact of menopause on the sexual health of Chinese cantonese women: a mixed methods study. J Adv Nurs. 2018;74:1672–84.

[37] Carvalho, A.O., Andrade, L.B., Ruano, F.F.L.O. et al. Knowledge, practices and barriers to access sexual health of women in the menopausal stages: a cross-sectional study with Brazilian gynecologists. BMC Women's Health 24, 52 (2024). https://doi.org/10.1186/s12905-024-02901-x

[38] Evcili F, Demirel G. Patient's sexual health and nursing: a neglected area. Int J Caring Sci. 2018;11:1282–8.

[39] American College of Obstetricians and Gynecologists' Committee on Practice Bulletins—Gynecology. Female sexual dysfunction: ACOG Practice Bulletin Clinical Management guidelines for Obstetrician-Gynecologists, Number 213. Obstet Gynecol. 2019;134:e1–18. https://doi.org/10.1097/AOG.0000000000003324

Another key barrier is the idea of discomfort, among professionals and perceived among patients. There exists a correlation between the level of knowledge of sexual health in aging women and the frequency in which it is addressed and discussed in the clinical setting[40]. This can develop and promote the proliferation of false assumptions on aging women. For example, a common one that still persists is that aging women are not sexually active.[41] In addition to the lack of clinical knowledge, a lack of knowledge on medical billing practices in relation to sexual health in aging women impacted the avoidance of treatment or guidance on certain issues. Age differences between health professionals and patients may also amplify this discomfort that is developed with a lack of knowledge on sexual health and ways to address it.

Broader Implementation of Telemedicine for Sexual Health Consultations in Rural United States

Cultural stigma about sexual health for aging women has exposed the Puritan views of American society which can lead to limited sexual health resources for aging women. For aging women, there is a belief that sexual desire and intimacy decrease as women age. According to Biomedcentral, a study concluded that "80% of men and 65% of women remain sexually active in old age [up to 74]".[42] Due to this misconception of sexual desire as well as menopause, there is an increased risk of sexually transmitted diseases like HIV due to a lack of access to safer sex supplies. According to the Centers for Disease Control and Prevention, "STIs have more than doubled in the past ten years among U.S. adults age 65 years and older" from 2007 to 2017 with

[40] Parish SJ, Clayton AH. Continuing medical education: sexual medicine education: review and commentary (CME). J Sex Med. 2007;4:259–68.
[41] Kuhle, C. L., Zhang, X., & Kapoor, E. (2021, April). Misconceptions about sexual health in older women: why we need to talk about it. In Mayo Clinic Proceedings (Vol. 96, No. 4, pp. 866-869). Elsevier.
[42] Banke-Thomas, Aduragbemi, Comfort Z. Olorunsaiye, and Sanni Yaya. 2020. "'Leaving No One Behind' Also Includes Taking the Elderly along Concerning Their Sexual and Reproductive Health and Rights: A New Focus for Reproductive Health." *Reproductive Health* 17 (1). https://doi.org/10.1186/s12978-020-00944-5.

a great increase in chlamydia, syphilis, and gonorrhea.[43] This is contrary to the increased production of contraceptives over the years, which means that stigma and lack of access to contraceptives and education are the driving forces of this disparity between younger and older generations. Older women have reported that there are limitations when it comes to discussing sexual health with healthcare professionals, educational materials, and contraception access.

Accessibility is especially an issue regarding geographic variances between rural and urban areas. Studies have discovered that the range of birth control options is less for women who live in rural areas compared to those who live in urban areas.[44] Additionally, there are reports that there were significant differences in sexual education for adolescents in rural areas which limits them to knowledge of contraception methods and potentially menopause. For example, there has been evidence of high rates of STIs within "Delta Regional Authority — 252 rural counties in the south that contain Alabama, Arkansas, Illinois, Kentucky, Louisiana, Mississippi Missouri, and Tennessee— and compared to non-Delta Regional Authorities the rates of chlamydia are higher by 75%, 131% for gonorrhea, and 112% higher for syphilis." [45] These discrepancies are due to various factors which include lack of access to contraceptives and education and stigma against sexual health especially for older women. This stigma is also exacerbated by the geographical distance that rural areas have compared to urban areas. For example, a study found that rural residents have to travel 17.8 miles for medical care, which is more than twice the distance that urban residents travel.[46] This can be detrimental for the aging population as it reduces access to a wide range of healthcare access, including sexual health

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[43] Smith, Matthew Lee, Caroline D. Bergeron, Heather H. Goltz, Tammy Coffey, and Ali Boolani. 2020. "Sexually Transmitted Infection Knowledge among Older Adults: Psychometrics and Test–Retest Reliability." *International Journal of Environmental Research and Public Health* 17 (7): 2462. https://doi.org/10.3390/ijerph17072462.

[44] Okwori, Glory, et al. "Geographic Differences in Contraception Provision and Utilization among Federally Funded Family Planning Clinics in South Carolina and Alabama." *The Journal of Rural Health*, vol. 38, no. 3, 6 Aug. 2021, pp. 639–649, https://doi.org/10.1111/jrh.12612.

[45] Valentine, J. A., Delgado, L. F., Haderxhanaj, L. T., & Hogben, M. (2021). Improving Sexual Health in U.S. Rural Communities: Reducing the Impact of Stigma. *AIDS and Behavior*, *26*, 1–10. https://doi.org/10.1007/s10461-021-03416-4

[46] Rural Health Information Hub. "Transportation to Support Rural Healthcare Introduction - Rural Health Information Hub." *Ruralhealthinfo.org*, 4 Dec. 2023, www.ruralhealthinfo.org/topics/transportation. Accessed 14 Apr. 2024.

consultation. Therefore, to provide privacy and more efficient access for those in rural areas, there should be a wider implementation of telemedicine that includes options to discuss sexual health with a certified healthcare provider.